

Cumulative Final Study Guide

EAM310 · East Asian Medicine Diagnosis I · VUIM Summer 2026

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Full Cumulative · Weeks 1–9, built from lecture transcripts + slides

How to use this guide: This is the deep-dive reference — the reasoning, analogies, and clinical stories behind every table, pulled directly from Schrier’s lectures. Read once per week for understanding, then drill the Cram Sheet for speed. Week 5 is intentionally absent (midterm week, no new content). Note: Weeks 7 and 8 topics are drawn from the transcripts that actually contain that content (Inquiry and Palpation respectively), since the raw file names for these two weeks are swapped relative to their actual topics.

1 • Week 1 — Vital Substances

The Vital Substances — Jing, Qi, Blood, Body Fluids, and Shen — are the raw material every later diagnostic system (8 Principles, Zang-Fu, Pulse, Tongue) reads for signs of excess, deficiency, stagnation, or heat. Schrier opens the course with these because you can't correctly name a pattern in Week 2 onward without first knowing what substance is out of balance.

The Three Treasures — Schrier's Candle Analogy

Jing, Qi, and Shen are called the Three Treasures, and the clearest way through them is the candle Schrier uses in lecture: **Jing is the candle itself** — the wax and the wick, the physical foundation. **Qi is the flame** — the energy in action, the process of burning. **Shen is the light the flame gives off** — the least material, the most refined, the actual purpose of the candle. You cannot have a flame without wax to burn, and once the wax is gone the flame goes out — which is why Jing is called the most superior of the three treasures: everything else depends on it. And you can't have light without a flame burning steadily, which is why Shen depends on both Jing and Qi being intact. This ordering (Jing → Qi → Shen, dense to refined) is the same ordering you'll see repeated everywhere in this system: physical substance, powering process, resulting phenomenon.

Treasure	Candle Role	Associated Organ/Element
Jing 精	Wax + wick (foundation)	Kidney, Earth
Qi 氣	The flame (energy in action)	Spleen/Stomach, Humanity
Shen 神	The light given off	Heart, Heaven

Jīng 精 (Essence) — The Bank Account Analogy

Pre-Heaven Essence is best understood as an inheritance: your parents' combined sexual energies at conception give you a fixed starting balance, stored in the Kidneys, that determines your constitutional strength for life. You cannot add to that original balance — once spent, it's gone. Post-Heaven Essence, extracted from food and fluid by the Spleen and Stomach after birth, works like income: it continuously deposits into the same account, topping up what the Pre-Heaven inheritance provided. Kidney Essence is the running balance of both combined.

Type	Source	Nature	Clinical Significance
Pre-Heaven 先天精	Blending of parents' sexual energies before birth	Fixed quantity/quality, cannot be replenished; stored in Kidney	Nourishes embryo/fetus; determines constitutional makeup, strength, vitality
Post-Heaven 後天精	Refined food & fluids after birth (Spleen/Stomach)	Can be stored and replenished	Continuously tops up Kidney Essence — "income" replenishing a fixed inheritance
Kidney Essence 腎精	Pre-Heaven + Post-Heaven combined	Fixed quantity; replenished by Post-Heaven; resides at Mìng Mén	Generates Tiān Guǐ (menses/sperm); produces Marrow → Spinal Cord → Brain

Quick Recall: *preserve Pre-Heaven Essence through good work/rest/diet/exercise habits — you can't earn more of it, only spend it slower.*

Jing Deficiency Signs	Detail
Children	Poor bone development, soft bones; late fontanelle closure; mental dullness/retardation; developmental delays
Adults	Softening of bones, weak knees/legs, lower backache, loose teeth; poor memory, absent-mindedness; tinnitus, deafness, premature greying; infertility, weakness of sexual activity

Qì 氣 (Vital Force) — What It Actually Means

Schrier is direct about a common complaint: people say Qi is impossible to define because it's "abstract." His counter is that Qi has a very specific set of meanings — the confusion comes from not being specific about which meaning is in play. Depending on context, Qi has been translated as vitality, vital breath, material force, matter-energy, or life force. The six contextual meanings that matter for this course: (1) vitality/energy/gas/air/vapor, (2) vital breath, (3) soul/creative force/inspiration, (4) aura, (5) environmental forces (the Liù Qì: Wind, Cold, Summer-Heat, Dryness, Fire, Dampness), and (6) nature/temperature (the Sì Qì: Hot, Warm, Cool, Cold).

Qi Formation Flow

Air Qi (inhaled by the Lungs) and Food Qi (extracted by Spleen/Stomach from what you eat and drink) combine in the chest to form Zong Qi (Gathering/Ancstral Qi). Zong Qi is the final raw material that becomes Zhen Qi (True Qi), which then splits into its Yin aspect (Ying Qi, nutritive, travels inside the vessels) and its Yang aspect (Wei Qi, defensive, travels outside the vessels). This is the same acupuncture Qi you're engaging when you needle a point.

Name	漢字	Source	Location	Key Functions
Congenital Qi	先天氣	Inherited at conception	Kidneys	Determines constitution; cannot be replenished; composite of Jing Qi & Yuan Qi
Air/Clear Qi	清氣	Inhaled by Lungs	Lungs	Inhalation and respiration
Food/Grain Qi	穀氣	Food → Stomach "rots & ripens" → Spleen transforms	Spleen/ Stomach	Origin of Qi & Blood; rises to chest to combine with Air Qi; Heart transforms into Blood
Gathering Qi	宗氣	Air Qi + Food Qi	Chest (stored)	Nourishes Heart & Lung; controls speech/voice strength; promotes blood circulation to extremities; regulates breathing with Yuan Qi
Source/Original Qi	原氣	Kidney Essence (Pre-Heaven)	Ming Mén; San Jiao	"Motive force" — arouses & moves all organs; conduit for San Jiao; exits at Source Points
True/Vital Qi	真氣	Zong Qi (final stage)	Meridians	Splits into Ying Qi & Wei Qi; this IS Meridian Qi
Nutritive Qi	營氣	Zhen Qi (Yin aspect)	Inside vessels/ channels	Nourishes internal organs; closely linked to Blood; the Qi activated during acupuncture needling
Defensive Qi	衛氣	Zhen Qi (Yang aspect)	Outside vessels; Còu Lǐ	Protects from pathogens; controls pores; moistens skin/hair; warms Zang-Fu; circulates 50×/24h (linked to Lungs)
Central Qi	中氣	Spleen Qi	Middle Jiao	Supports transformation/transportation; raises Qi (prevents prolapse) — "Earth holds everyone in check"
Upright Qi	正氣	Wei Qi + Ying Qi + Kidney Essence	Whole body	Body's total defense; opposite of Xié Qì (pathogenic Qi)

6 Functions of Qi

Schrier frames movement as the core principle behind all six functions: "Life is about movement. If there is proper movement, there is no pain. If there is no movement, there is pain in the body."

Function	Mechanism	Clinical Note
Transporting	Movement in all directions: up/down (ascending/descending), in/out (entering/exiting). Think of Qi as railways moving information, goods, and troops through a city.	No movement = pain
Warming	Function of Yang Qi — all physiological processes depend on warmth	Fluids are Yin and need Yang warmth to transform/transport/excrete; Qi def → Cold signs
Protecting	Primarily Wei Qi, nourishing the Còu Lǐ (space between skin and muscles — the exterior energetic layer)	The Lungs govern the skin, the most exterior part of the body; Wei Qi strength = resistance to Wind/Cold/Damp
Holding	Yang Qi holds Yin substances (Blood, fluids) in their proper place	Deficiency → incontinence, excessive bleeding/sweating — things "leak out"
Transforming	Converts dense matter (food, fluids) into subtler Qi forms	Qi def → Phlegm & Dampness accumulation
Rising	Holds body structures in proper anatomical position; primary organ is Spleen (Zhōng Qì)	Qi def → sinking → organ prolapse (uterus, rectum, stomach); distinct from Holding (which is anti-leak, not anti-sink)

Qi Pathologies — 4 Patterns

Pattern	Mechanism & Cause	Signs & Symptoms	Tongue/Pulse
Qi Deficiency	More Qi consumed than produced. Affects Lung, Spleen, Kidney, Heart. Causes: illness, overwork, poor diet, lack of sleep, excessive exercise.	Fatigue (key symptom, both physical and mental), slight SOB, weak voice, spontaneous sweating, poor appetite, loose stools	Empty pulse; pale/normal tongue
Sinking Qi	Aspect of Qi Deficiency — insufficient Yang Qi to rise and ascend. Primary organ: Spleen.	All Qi Def signs plus: feeling of bearing down, mental depression, listlessness, organ prolapse	Empty pulse
Qi Stagnation	Often Liver, also Lungs/Heart/Stomach/Spleen/Intestines. Causes: emotional stress, irregular eating, excessive work.	Distension (subjective/objective) — more distension than pain — moving/distending pain, mental depression, irritability, frequent sighing, mood swings	Wiry pulse; tongue normal or slightly red on sides
Rebellious Qi	Qi moves in the wrong direction for that organ (Maciocia Table 31.1)	Stomach should descend — ascending=belching/nausea/vomiting. Spleen should ascend — descending=diarrhea/prolapse. Liver excessive ascent=headache/dizziness/irritability; excessive horizontal=nausea/belching (invading Stomach) or diarrhea (invading Spleen). Lung/Kidney should descend — ascending=cough/asthma. Heart should descend — ascending=restlessness/insomnia.	Varies by organ

Clinical judgment call from lecture: when a patient says "I'm depressed," Schrier stresses you can't stop there — "mental depression" appears in both Sinking Qi and Qi Stagnation patterns. You have to unpack what the depression actually feels like (heavy/listless = more Sinking; irritable/gloomy with mood swings = more Stagnation) before you can commit to a pattern.

Xuè 血 (Blood)

Blood in East Asian Medicine is not the Western hematological fluid — it's described as a very dense, material form of Qi, the most Yin of the vital substances. Sū Wèn 26 states "Blood is the mind of a person," and Líng Shū 32 adds that "when Blood is harmonized, the mind is calm in its residence." Schrier frames the Yin-Yang split along gender lines as a teaching heuristic: men tend to be more ruled by Qi, women more by Blood — which is also why Blood patterns show up so prominently in gynecological presentations (menstruation, Tiān Guǐ).

Source Organ	Role in Blood Formation
Spleen	Main source — generates Blood from Gǔ Qì (Food Qi)
Lung	Pushes Gǔ Qì toward the Heart for transformation
Heart	Transforms Gǔ Qì into Blood
Kidney	Makes Blood via Marrow and Kidney Essence

#	Function of Blood
1	Nourishes all tissues and organs
2	Circulates with Yīng Qì (inseparable)
3	Moistens body, skin, sinews, eyes
4	Houses the Shen — anchors the Mind; Blood deficiency → insomnia, anxiety
5	Determines Menstruation (via Tiān Guǐ)

Quick Recall: the three organs most closely engaged with Blood are Heart (governs circulation), Spleen (makes it, keeps it in the vessels), and Liver (stores it, nourishes sinews, moistens eyes, regulates menstruation).

Blood Pathologies — 4 Patterns

Pattern	Mechanism	Signs & Symptoms	Tongue/Pulse
Blood Deficiency	Fails to nourish/moisten; fails to anchor Shen. Causes: inadequate blood-rich foods, deficiency in blood-producing organs, blood loss, consumption by excessive mental/physical activity.	Dull-white sallow complexion, pale lips, dizziness, poor memory, numbness/tingling, blurred vision, insomnia, scanty periods/amenorrhea, depression, slight anxiety	Pale, thin, slightly dry tongue; Choppy or Fine pulse
Blood Stasis	Potentially serious — linked to cancer, fibroids, endometriosis, heart disease, stroke. Causes: Qi stagnation (most common — "Blood is the mother of Qi, so if Qi stagnates, Blood stagnates"), Qi deficiency, Heat in Blood, Blood deficiency, Interior Cold, Emotions, Diet, Trauma.	Stabbing, fixed, or boring pain; dark complexion; purple lips and nails; dark blood with dark clots; fixed abdominal masses	Purple tongue (location varies); Wiry, Choppy, or Firm pulse
Heat in Blood	Invasion of Heat to the Yin/Blood level, OR excess internal heat in Liver/Heart	Feeling of heat, skin diseases with red eruptions, thirst, bleeding. Heart→anxiety/mouth ulcers; Liver→skin itching/redness; Uterus→excessive blood loss; Intestines→blood in stools	Red tongue; Rapid pulse
Loss of Blood	Cause 1: Qi Deficiency → cannot hold Blood (deficiency pattern). Cause 2: Blood Heat → pushes Blood out (excess pattern). Less common: Blood Stasis, Yin Deficiency.	Bloody nose (Lung), vomiting blood (Stomach), coughing blood (Lung), blood in stool (Large Intestine), menorrhagia/metrorrhagia (Liver/Uterus), blood in urine (Bladder)	Pale tongue; Fine/Choppy pulse

Schrier offers two memorable images for the mechanisms behind Blood Stasis: Interior Cold congealing Blood is like putting liquid in a freezer — it makes ice. Heat condensing Blood is like heating an instant gravy packet on the stove: keep the heat on and keep stirring, and eventually it thickens into a brick if you leave it too long.

Qi Stagnation vs Blood Stasis — Key Differentiator (Maciocia Table 32.2)

	Qi Stagnation	Blood Stasis
Pain vs distension	More distension than pain; moving	More pain than distension; fixed
Pain character	Distending, comes & goes with emotional state	Boring or stabbing, constant
Abdominal masses	Appear and disappear	Fixed
Skin	Not visible on skin	Purple blotches or bruises
Tongue	Normal or slightly red on sides	Purple on sides (or location-specific — see below)
Pulse	Wiry	Wiry, Firm, or Choppy

Blood Stasis — Signs by Location

Schrier's teaching device for remembering how Blood Stasis spreads to a paired organ: think of a messy roommate. The mess is contained in their room at first, but over time the smell lingers into the shared space, and eventually the whole apartment is affected. Similarly, Stomach Blood Stasis can show resonance in the Large Intestine (its paired Yang Ming organ) because "when one thing happens to one, you might see similar resonances in its paired organ."

Location	Key Signs
Liver	Purple nails (Liver manifests on nails); dark face; painful periods with dark clots; abdominal & premenstrual pain; purple tongue on the sides; wiry/firm pulse
Heart	Purple lips; stabbing/pricking chest pain; mental restlessness (Shen disturbance); purple tongue toward the front (chest region); distended sub-lingual veins; choppy/knotted pulse
Lung (Upper Burner)	Chest oppression; coughing dark blood; tongue purple on sides toward the front
Stomach (Middle Burner)	Epigastric pain; vomiting dark blood; tongue purple in the center (Stomach's tongue zone)
Intestines/Uterus (Lower Burner)	Dark blood in stools; severe abdominal pain; painful periods with dark clots; amenorrhea; fixed abdominal masses; wiry/firm pulse

Jīn Yè 津液 (Body Fluids)

Fluids originate from food and drink and are separated by the Spleen into clean and turbid. Clean fluids rise to the Lungs and spread to the skin, then descend to the Kidneys. Turbid fluids go to the Small Intestine for a further round of separation — pure fluids continue to the Bladder (becoming sweat via the exterior), impure fluids continue to the Large Intestine (becoming urine via the downward path). Schrier's point here is philosophical as much as clinical: "even within the turbid, there is a clear; within the clear, there is a turbid" — Yin and Yang are infinitely divisible, so even the "impure" fraction sent toward the Large Intestine still separates further into its own pure/impure gradient.

	Jīn 津 (Yang)	Yè 液 (Yin)
Character	Clear, light, thin, watery	Heavy, dense, turbid
Circulates with	Wei Qi, on the Exterior (between skin/muscles)	Ying Qi, in the Interior
Movement	Quick	Slow
Controlled by	Lung (Upper Burner)	Spleen & Kidney (Middle/Lower Burner)
Moistens	Skin & muscles — manifests as sweat, tears, saliva, mucus; becomes part of Blood	Joints, spine, brain, bone marrow; lubricates orifices (eyes, ears, nose, mouth)

Quick Recall (mnemonic from lecture): "Jin" like gin the drink — clear and thin. Fluid Deficiency is Yin in nature, so deficiency always presents as dryness.

Edema and Phlegm — Reading the Fluids Excess Side

Excess fluid patterns (edema, phlegm) mirror the deficiency patterns above but in reverse. Pitting edema (finger leaves a dip that takes time to disappear) is a Yang deficiency of Spleen, Lung, and/or Kidney — the Yang can't mobilize fluids so they pool. The location where edema starts is diagnostic: starting in the face/hands and spreading = Lung Yang deficiency; starting in the abdomen spreading to the limbs = Spleen Yang; starting in the legs/ankles = Kidney Yang. Non-pitting edema points instead to Qi stagnation or Dampness.

Phlegm Type	Presentation
Damp-Phlegm	Profuse sticky white phlegm, chest/epigastric oppression, nausea, sticky taste, no thirst; swollen tongue w/ sticky coat; slippery pulse
Phlegm-Heat	Sticky yellow phlegm, chest oppression, red face, dry mouth, restlessness; red swollen tongue w/ sticky yellow coat; rapid slippery pulse
Cold-Phlegm	White watery phlegm, chest oppression, cold limbs, nausea; pale swollen tongue w/ white wet coat; deep slow slippery pulse
Wind-Phlegm	Dizziness, nausea, vomiting, numbness (esp. unilateral), coughing phlegm, rattling throat sound, aphasia; swollen deviated tongue; wiry pulse

Substantial phlegm (the sputum you can see and cough up) is distinct from non-substantial phlegm, which is retained subcutaneously or in the channels — this is the phlegm that can obstruct the Heart orifice and block the Shen, or settle in joints causing arthritic deformities, or in the Gallbladder/Kidneys as stones.

Shén 神 (Spirit) — Healthy vs. Deranged

Returning to the candle: you can hold the wax (Jing) and even touch the flame (Qi, though Schrier doesn't recommend it), but you cannot grasp the light itself. Shen is the least material of the Three Treasures, and describes the whole mental, emotional, and spiritual aspect of a person — not just "spirit" in a narrow sense.

Observation	Having Shen	Loss of Shen	False Shen
Complexion	Bright and moist	Dim, dull sheen	Dull, then suddenly lustrous
Eyes	Alert, vivid, twinkle	Dull, blurred vision	Dull, then suddenly bright
Mind/Speech	Clear speech, agile	Confused, incoherent	Unclear, then sudden change
Respiration	Smooth, even	Uneven, shallow, SOB	Feeble, then briefly steady

False Shen is a warning, not a recovery: a chronically ill, exhausted patient can suddenly perk up — alert eyes, strong clear speech — and then pass away shortly after. This is a sudden floating-up of Yang Qi as Yin and Yang begin to separate at the end of life, not genuine improvement.

Deranged Shen Patterns

Term	Presentation
Fán zào	Irritability with hot/feverish sensation; may drive the patient to prefer nudity or jump into water
Zàng zào (Lily disease pattern)	Mental depression, hallucination, disorientation, frequent crying spells, bouts of yawning
Bǎi hé bìng	Chronic depression, apathy, aversion to speaking/mumbling to self, insomnia, anorexia, sleepwalking
Diǎn (Yin madness)	Chronic depression, aversion to speaking or mumbling to self, inappropriate laughing/crying
Kuáng (Yang madness)	Raving with obscenities, restlessness, singing loudly in high places, wandering naked, aggression

The Five Spirits (Wu Shen) — One per Zang Organ

Organ	Spirit	Domain	Imbalance Signs	Outer BL Point
Heart	Shén (small-s)	Consciousness, insight, awareness, compassion; aligns one to the world and allows communication; reflected in the brightness of the eyes	Mania, lack of inspiration, scattered spirit, insomnia, hysteria, sadness, depression	Mind Hall (outer BL, across from Heart Back-Shu) — anxiety, insomnia, depression
Liver	Hún 魂	Vision, imagination, direction, benevolence; the "soul" that enters at birth and continues after death; repository of dreams, plans, aspirations	Delusions, hallucinations, psychosis, nightmares, sleepwalking/talking, anger, frustration, jealousy, apathy, paranoia	Gate of the Ethereal Soul — roots and steadies the Hun, aids capacity for life planning
Lung	Pò 魄	Animal wit, embodiment, bodily sensation; dies when the body stops functioning; gives instinctive reactions and the ability to feel sensations	Emotional numbness, mind-body disconnect, overindulgence, obsession with beauty/vanity, sadness, grief, inability to let go	Gate of the Corporeal Soul — soothes spirit, nourishes Qi, aids letting go of prolonged grief
Spleen	Yì 意	Integrity, intention, clear thought, devotion; ability to think clearly, study, and manifest ideas; "standing on firm ground"	Poor concentration, lack of expression, overthinking/worrying, stuck thought patterns, lack of sympathy or over-smothering	BL 49 — tonifies mental aspects of Spleen: memory, concentration, study capacity
Kidney	Zhì 志	Willpower, courage, drive, survival instinct; the fundamental push and determination to be alive	Lack of willpower/drive, no life goals, depression, fearfulness, low self-esteem	BL 52 — strengthens willpower and determination

Substance Relationships — Statements of Fact

Relationship
Qi is the commander of Blood; Blood is the mother of Qi.
Blood nourishes Body Fluids; Body Fluids replenish Blood.
Qi transforms, transports, and holds Body Fluids; Body Fluids help nourish Qi.
Liver Blood nourishes Essence; Kidney Essence transforms into Blood.

2 • Week 2 — Eight Principles 八綱

Before the Eight Principles, Schrier reframes what "disease" (病, bing) even means in this system: it is not a fixed Western diagnostic label but an interruption of the smooth flow of Qi through the body, described through a distinct set of signs, symptoms, and pathology. His standing answer when a student asks "what's the best acupuncture point for lupus?" is "I don't know what lupus is — what is your TCM diagnosis?" The system doesn't treat disease names; it treats patterns, and the Eight Principles are the first and most fundamental sorting mechanism for identifying one.

Why Pattern, Not Cause, Comes First

Schrier is explicit that presenting signs and symptoms should not be mistaken for the cause of disease. A patient with loose stools, tiredness, and no appetite tells you they have Spleen Qi Deficiency — that's the pattern. What's causing the Spleen Qi Deficiency (mental rumination, worry, an irregular diet, too much greasy/dampening food) is a separate question you ask afterward. Herbs and acupuncture can address the pattern, but if you don't help the patient adjust the underlying cause, the pattern will keep recurring. The same logic applies to disease-vs-pattern mapping in general: one disease (e.g., painful periods) can result from several different patterns (Qi stagnation, Blood stasis, Cold in the Uterus, Damp-Heat in the Uterus), and one pattern can show up as several different Western "diseases." The Eight Principles, Qi/Blood/Fluids, and Six Conformations are the diagnostic layer; treatment principles (sweating, vomiting, purging, harmonizing, warming, clearing, dispersing, supplementing) come after.

Yin/Yang — The Master Pair

Pair	Characters	Key Differentiator
Yin/Yang	陰陽	Master pair. Yin = Interior+Cold+Deficiency (slower, quieter). Yang = Exterior+Heat+Excess (faster, louder, active).

Schrier ties Yin/Yang directly back to Blood and Qi from Week 1: Yang is energy (Qi), Yin is fluid (Blood). As a teaching heuristic (not a clinical rule), men tend to be more ruled by Qi (more Yang), women more by Blood (more Yin) — which is one reason gynecological patterns lean so heavily on Blood pathology.

Excess vs. Deficiency — The Belly-Push Test

Schrier's clinical image for Excess vs. Deficiency: press on a distended, excess-type belly and the patient winces — "ooh, that hurts" — because it's already full and pressing makes it worse, like poking an overinflated balloon. Press on a deficient patient's belly and giving that pressure (or that added Qi, that added energy) feels good to them — "ooh, that feels so good" — because you're supplementing something that was lacking. This is the single differentiator Schrier tells students to hang onto: **pressure makes Excess worse and makes Deficiency feel better.**

Pair	Characters	Key Differentiator
Deficiency/Excess	虛實	Pressure response is the ONE key differentiator. Deficiency = pain RELIEVED by pressure/warmth. Excess = pain AGGRAVATED by pressure.

Cold vs. Heat — the Ice Water and Gravy Packet

Schrier links Cold/Heat straight back to the Blood Stasis mechanisms from Week 1: cold is like putting liquid in the freezer — it congeals into ice. Heat is like heating an instant gravy packet on the stove: keep stirring over heat and it thickens, and if you leave it too long "it becomes a brick." Full Heat (Excess Yang) makes heat rise — redness, headache, agitation, irritability, because "heat is Yang, and Yang rises." Deficiency of Yang is Empty Cold: when Yang-warmth is gone, cold moves in — desire for warmth, cold limbs, pale urine. Excess of Yin produces the same cold-body picture from the opposite direction (too much of the cold, heavy, sinking substance itself, rather than not enough warmth to counter it).

Pair	Characters	Key Differentiator
Cold/Heat	寒熱	Full Heat = whole face red, thick yellow coat, strong Rapid pulse. Empty Heat = malar flush only, little/no coating, thin Rapid pulse.

Pattern	Mechanism	Key Signs
Full Heat (Excess Yang)	Excess Yang rising	Red face, headache, agitation, irritability, thirst for cold, thick yellow coat, strong Rapid pulse
Empty Heat (Yin Deficiency)	Not enough Yin/fluid to balance normal Yang — heat by default rather than by excess	Malar flush (not whole face), night sweats, five-palm heat, dry throat worse at night, thin Rapid pulse
Full Cold (Excess Yin)	Excess of the cold, heavy Yin substance itself	Strong aversion to cold, cold limbs, may still have some strength/tension present
Empty Cold (Yang Deficiency)	Not enough Yang-warmth to counteract normal Yin	Desire for warmth, cold limbs, pale urine, weakness alongside the cold (this is essentially Qi Deficiency + Cold)

Quick Recall: whether it's Excess Cold or Deficient Yang, the body wants warm things. Whether it's Excess Heat or Deficient Yin, the body wants cool things — the craving tells you the temperature regardless of which mechanism produced it.

Exterior vs. Interior — Don't Trust the Pulse Alone

Schrier's example of inductive vs. deductive reasoning centers on this pair. Deductively: fever + aversion to Wind-Cold + aching body + stiff neck + Floating pulse → you can generally assume Exterior. But that's an assumption built on the typical case (inductive reasoning from historical pattern), not a guarantee. A Floating pulse can also show up in Yin Deficiency — and if it's Yin Deficiency, the accompanying picture is completely different: dryness, fever at night, night sweats, five-palm heat, malar flush (not whole-face redness). The floating pulse alone is a question mark, not an answer — you always need the accompanying signs before committing to Exterior.

Pair	Characters	Key Differentiator
Ext/Int	表里	Exterior = fever + aversion to cold SIMULTANEOUSLY + Floating pulse. Lose the pairing → moved Interior (Deep pulse).

Schrier also flags a related trap: a patient can present with cold limbs but a hot torso. That is not simply "a cold pattern" — it's more likely internal stagnation blocking Qi from reaching the extremities to warm them, which is why the limbs are cold even though there's heat trapped in the core. Read the whole pattern, not the first symptom that seems to fit.

Shao Yang — the "Katy Perry / Hokey Pokey" pattern: Schrier's mnemonic for the half-Exterior, half-Interior stage between Tai Yang (aversion to cold, fever radiating off) and Yang Ming (aversion to heat): the patient is "hot and cold, yes and no" — in and out, like the hokey pokey, or Katy Perry's "Hot N Cold." This is a useful bridge concept between the 8 Principles and the Six Conformations you'll see built on top of them later in the program.

Why 8 Principles come first: every other diagnostic system (Qi/Blood/Fluids, Zang-Fu, Meridians, Six Conformations, Four Levels, Five Elements) presumes you've already correctly sorted Excess vs. Deficiency and Hot vs. Cold. Get it wrong and, as Schrier warns with the erectile-dysfunction example from his own practice, you can tonify Yang in someone who's actually Yin-deficient and Heart-based rather than Kidney-based — burning them up further instead of treating the actual pattern.

Critical Thinking Modes in Pattern Differentiation

Mode	How It Works
Deductive	From presenting signs/symptoms to a general pattern (fever+aversion to cold+stiff neck+Floating pulse → likely Exterior)
Inductive	General conclusions from historical/typical cases (most patients with a Floating pulse have an Exterior pattern) — useful, but not always true; always check the accompanying signs
Analogical	Reasoning by known mechanism-relationships (Heat is known to generate Wind over time → if a patient has long-term internal Heat, expect an eventual internal Wind pattern)

3 · Week 3 — Pulse Diagnosis 脈診

The character 脈 (mài) combines the flesh radical with a component suggesting something long, perpetual, and always-present — the pulse as a permanent feature of the body. Schrier draws out the moon connection: blood is liquid, the moon governs tides, and menstruation has its own lunar rhythm — "the moon is a mirror for the sun's light, the way the pulse is a mirror that lets us observe Yin and Yang, since the shape of the pulse changes and possesses all the essential Yin and Yang." This is the conceptual reason pulse diagnosis carries so much diagnostic weight: it's a direct readout of the Yin-Yang state of the whole body.

Handling Subjectivity Honestly

Schrier is upfront about the central weakness of pulse diagnosis: it's extremely subjective. He tells a clinic story where he, his practice partner, and their supervisor all felt different things on the same patient's pulse — the supervisor's explanation was that the patient may not have felt comfortable with the more forceful, "nosy" partner, and that discomfort itself changed the pulse. His broader point: state changes the pulse. A patient who just ran up the stairs to make their appointment will show a temporarily Rapid pulse that reflects nothing but exertion, not an underlying Heat pattern. This is why Schrier's teaching method emphasizes asking about accompanying signs and symptoms (thirst, sweating, tongue color) rather than trusting a single pulse reading in isolation.

The practical fix for this subjectivity, per the Ling Shu, is presence: "shut the door and close the windows and attune yourself directly with your patient" — meaning no outside distraction, full attention (Yi, the Spleen's spirit of intention) on the patient in front of you, because "Qi follows Yi." Schrier also uses a fishing analogy for pulse-taking patience: you cast your line and the fish doesn't always bite immediately. If you can't find a quality right away, sit still, stay patient, and clear your own mental agitation — shaking the line and demanding the fish bite doesn't work any better in pulse-taking than it does in fishing.

Six Positions

Pulse positions follow Nan Jing Difficulty 18 (verified against FCM Ch. 30). Schrier notes the Nan Jing is where radial pulse diagnosis really gets standardized (Han Dynasty) and maps more directly onto channel theory, versus Li Shizhen's later system which leans more toward herbal medicine framing — a useful distinction, though he cautions it's not an absolute rule. Each position reflects a Zang + paired Fu together, never a single organ.

Position	Left Hand	Right Hand
Cùn (superficial)	Heart / Small Intestine	Lung / Large Intestine
Guān (middle)	Liver / Gallbladder	Spleen / Stomach
Chǐ (deep)	Kidney / Bladder	Pericardium / San Jiao

Trap: Right Chǐ is Pericardium/San Jiao specifically, linked to Kidney only via Ming Men theory — NOT Kidney-Yang/Ming Men directly. Schrier also notes some schools of thought (e.g., Li Shizhen) don't even consider the state of the Yang organs at each position — different teachers, different systems; ask which system a source is using before assuming a discrepancy is an error.

Method & Finger Technique

The three finger movements Schrier physically demonstrates: **Lifting** (light contact at the superficial level — "you make contact, press down just gently"), **Pressing** (press all the way down to the bone, obliterate the pulse until you feel nothing, then release very gradually until something reappears under your fingertips — this is how you find the deep level), and **Searching/Rolling** (moving the fingertip left-right like an old joystick to assess length and width, since a static finger only tells you rate and rhythm).

Concept	Detail
5 finger movements	Lifting (superficial) · Pressing (deep) · Searching (rate) · Pushing (shape) · Rolling (length)
Normal pulse 胃神根	胃 Wèi = flowing/gentle · 神 Shén = strong/rhythmical · 根 Gēn = rooted, deep at Chǐ
Normal rate	4–5 beats per practitioner's breath, 60–90bpm
8 components	Width, Depth, Rate, Rhythm, Strength, Length, Tension, Shape

Seasonal Pulses — Schrier’s Images for Each

Season	Pulse	Dates	Schrier’s Image
Spring	Wiry 弦	Feb 4 – May 4	Tense things starting to burst out of the ground — life pushing to shoot outward, trying to find the strength to break through.
Summer	Flooding 洪	May 5 – Aug 6	Yang Qi still rising and spreading, uninhibited — "a bird spreading its wings and soaring away." A pulse you don't want to see this strongly in Winter.
Autumn	Hair-like 毛	Aug 7 – Nov 6	Soft, dainty, floating but gentle — it doesn't "meet you" intensely the way a true Exterior Floating pulse does. You feel it at the top only.
Winter	Stone 石	Nov 7 – Feb 3	Like throwing a stone into a well — it sinks and ripples, and finding it again takes real effort. Deep and hard to locate.

The cerebral hemorrhage case: Schrier recounts a teacher, Dr. Lili Hong, who found a strong Flooding pulse on a patient in *Winter* — already inappropriate for the season. He warned the patient and prescribed herbs; the patient didn't take them. When Summer arrived, the Yang Qi that was already excessive in Winter had nowhere left to build except further, "like a volcano erupting" — the patient suffered a cerebral hemorrhage and died. The lesson: a seasonal pulse is only "normal" when it matches its season and has no accompanying abnormal signs. The same Flooding quality showing up out of season is a genuine warning sign, not a curiosity.

Quick Recall: don't assume a seasonal-quality pulse is automatically "just the season" — check for the corresponding symptoms (frustration/stagnation signs for a Wiry Spring pulse, Heat signs for a Flooding Summer pulse) before writing it off as normal.

Pulse Blocks — The Hose Analogy

When Qi flow through the channels hits a block (often at Yin/Yang meridian transition points, the entry-exit points of the Ying cycle), Schrier compares it to pinching a garden hose: the water backs up and feels full/tight right at the pinch point, then comes out weak and thin past the blockage. The clinical response is to needle the exit point of the blocked channel and the entry point of the next channel in the cycle to release the pinch and let Qi move smoothly again — the same logic as un-kinking the hose so you can water your flowers properly.

Core Combinations

Base Quality	+ Second Quality → Pattern
Floating +	Tight=Ext Wind-Cold · Rapid=Ext Wind-Heat · Empty=Summer-Heat or Yin Def · Slippery=Wind-Damp/Phlegm · Weak=External-Empty
Deep +	Slow=Int Cold · Rapid=Int Heat · Weak=Yang Def · Wiry=Blood Stasis/Chong Mai · Soggy-Slow=Int Dampness
Slow +	Weak=Empty-Cold/Yang Def · Full=Full Cold (chronic pain) · Choppy=Blood Def+Int Cold · Slippery=Cold-Phlegm · Wiry=Cold in Liver channel
Rapid +	Full=Full Heat · Empty=Empty Heat (Yin Def) · Wiry=Liver Fire · Slippery=Phlegm-Heat · Overflowing=Full Heat

The Traps

Trap	Detail
Floating ≠ always Exterior	Also Yin Deficiency — Yin fails to anchor Yang. Presents Floating+Thin+Rapid, night sweats, five-palm heat, malar flush, dry throat worse at night. True Exterior floating has SIMULTANEOUS aversion to cold + fever.
Huǎn is the healthy one	Slowed-Down/Moderate 緩 = 4 beats/breath, faster than Slow = the only qualitatively normal entry among the 28. Easy to misread as Slow.
Irregular rhythm 3-way split	Knotted 結 = SLOW+random stops → Cold w/ Heart-Yang Def. Hasty 促 = RAPID+random stops → Severe Heat/Heart-Fire. Intermittent 代 = pauses at REGULAR intervals → most serious, internal organ/Heart problem.

All 28 Pulse Qualities — With Tactile Images

Schrier’s teaching device here is the same one used for recognizing people: "what does a bank robber look like? A police officer? A firefighter?" — you build a mental image for each pulse quality the same way you'd recognize a family member's personality. The tactile analogies below are the ones used in lecture and lab to build that recognition.

#	Quality	Tactile Image	Indication
1	Floating 浮	A subtle breeze across a bird's back	Invasion of external Wind; Yin Deficiency presenting superficially
2	Deep/Submerged 沉	A stone sinking to the bottom of a well	Pathogenic factor in the Interior; Yang Deficiency (Deep-Weak)
3	Slow 遲	Fewer than 3 beats/breath, ~40–60bpm	Cold pattern (Empty-Cold if Weak, Full-Cold if Full)
4	Rapid 數	Over 5 beats/breath, 90–140bpm	Heat pattern (Full-Heat if Full, Empty-Heat if Floating-Empty)
5	Empty/Deficient 虛	A water balloon — big outside, nothing solid on pressure	Qi Deficiency
6	Full/Excessive 實	Felt at all 3 positions and all 3 depths	Full pattern
7	Slippery 滑	A pearl rolling on a plate	Phlegm, Dampness, food retention, pregnancy
8	Choppy 澀	Scraping bamboo with a small knife	Blood Deficiency; exhaustion of Body Fluids
9	Long 長	Smooth and straight, extends beyond its normal position	Heat patterns
10	Short 短	Cannot be felt in all 3 positions	Severe Qi Deficiency; Stomach-Qi Deficiency
11	Overflowing 洪	Ocean waves — come strongly, leave calmly	Full-Heat (Overflowing-Empty = Empty-Heat)
12	Fine/Thin 細	Angel-hair pasta, not thick udon	Blood Deficiency; Dampness with severe Qi Def
13	Minute/Faint 微	Now you see it, now you don't	Severe deficiency of Qi and Blood
14	Tight 緊	A twisted cord snapping back and forth	Cold (Ext, Int Full, or Int Empty by depth); Pain
15	Wiry 弦	A guitar string about to snap	Liver disharmony; Pain; Phlegm
16	Slowed/Moderate 緩	4 beats/breath — faster than Slow	Generally a HEALTHY pulse — the one normal entry
17	Hollow 芤	A scallion stalk — edge but hollow inside	Hemorrhage; forthcoming hemorrhage if Rapid; Yin Def
18	Leather 革	A drum surface — thick on top, hollow beneath	Severe Kidney-Essence/Kidney-Yin Deficiency
19	Firm/Confined 牢	Deep, full, and unmoving — found only by pressing hard	Blood Stasis; Interior Cold (if Slow); Pain
20	Soggy 濡	Floating, fine, and soft — disappears with heavier pressure	Dampness with Qi Deficiency; Yin/Essence Deficiency
21	Weak/Frail 弱	Deep, fine, and soft	Yang Deficiency; Blood Deficiency
22	Scattered 散	Diffuse, without a clear root at any depth	Severe Qi and Blood Deficiency; serious condition
23	Hidden 伏	Deeper than Deep — found only by pressing to the bone	Severe Yang Deficiency
24	Moving 動	A short, jumping, bean-like beat felt at one position	Shock, anxiety, fright; Pain
25	Hasty 促	Rapid + random stops	Severe Heat; Heart-Qi Deficiency; Heart-Fire
26	Knotted 結	Slow + random stops	Cold with Heart-Yang Deficiency
27	Intermittent 代	Pauses at regular intervals	Serious internal organ / Heart problem
28	Hurried/Racing 疾	Faster even than Rapid	Fire exhausting Yin

4 • Week 4 — Tongue Diagnosis 舌診

Schrier draws a direct contrast between pulse and tongue that's worth holding onto for the whole topic: Qi is Yang — active, moving, quick to shift — so the pulse can change noticeably within a single treatment or even a single stressful moment. Blood is Yin — heavier, more stable, slower — so the tongue "doesn't lie" the way a pulse can be thrown off by someone running up the stairs to their appointment. If you needle someone well, you won't usually see the tongue shift much in that same session (maybe a little more moisture if it was dry), whereas the pulse might shift right there on the table. This is part of why tongue and pulse are meant to be read together rather than either one alone — and why, as Schrier notes, telemedicine only gives you one weak leg of the diagnostic stool: no pulse, no abdominal palpation, just a flat video image of the tongue.

Don't Clean Up the Crime Scene

Schrier's memorable warning: the tongue body color always shows the true condition, but that's only useful if you haven't disturbed the evidence. Scraping your tongue right before an appointment is, in his words, like "cleaning up the crime scene" — wiping away the fingerprints before the detective arrives. Chewing a Jolly Rancher, drinking Mountain Dew or Code Red, or having coffee beforehand can all distort color and coating temporarily. None of this means the patient is lying to you; it means you ask before you conclude — "did you have coffee this morning?" before committing to a Heat diagnosis off a yellow coating alone.

How to Inspect

Schrier disagrees with Maciocia's textbook timing of 15–20 seconds for tongue protrusion — he teaches 10 seconds maximum, because a tongue held out longer starts to quiver from muscular fatigue, which can be mistaken for a pathological tremor. His practical technique: short blurbs. "Stick out your tongue. Great, thank you, you can put it back in. Stick out your tongue again, thank you, put it back in, rest." This is easier on the patient and yields cleaner, less-strained images. He also notes the social awkwardness of the exam — most of us were taught as children that sticking your tongue out at someone is rude — and suggests defusing it with a light joke ("remember when your parents said not to do this? Now you get to.").

Aspect	Reflects
Body color	Blood/Nutritive Qi; Heat/Cold and Yin/Yang of the Yin organs
Body shape	State of Blood/Nutritive Qi; moisture; excess or deficiency
Coating	State of the Yang organs; Heat/Cold; Fullness/Deficiency
Moisture	State of the Body Fluids
Spirit	Does the tongue look alive and vibrant, or lifeless — like a shriveled raisin

Medications and Other Distortions

This is board-exam material Schrier flags explicitly. Antibiotics create a peeled coating that appears within a couple of days of starting the medication and takes about two weeks after stopping to return to normal — asking "are you taking any antibiotics?" when you spot this pattern is a reliable way to "blow a patient's mind."

Medication	Tongue Effect
Antibiotics	Peeled coating (appears within days, resolves ~2 weeks after stopping)
Corticosteroids	Red, swollen tongue; redder tip
Bronchodilators	Red tip only, after many years of use (Lung correspondence — front third of tongue)
Diuretics	Peeling
Anti-inflammatories	Red points; thin body
Antineoplastics (chemo)	Thick, brown/black, dry coating
Pepto-Bismol	Black tongue coating

Quick Recall: age and gender matter too — children run hot (more Yang, often a redder tongue, not necessarily pathological); a person may show a paler tongue right after menstruation (Blood loss) and a redder tongue (especially on the sides, Liver zone) just before it starts.

Tongue Zang-Fu Map — and Why Maps Disagree

Schrier is candid that tongue maps vary by textbook, school, and country — "buy a map of Virginia from 2015, then one from 2026, and there will be some new roads" but the overall shape holds. His advice when a source disagrees with what you learned: don't panic, work from the burner-level zones you're confident about (front=Upper Burner, center=Middle Burner, rear=Lower Burner) and then narrow down using the accompanying clinical signs rather than the map alone. If a patient has clear bowel signs (constipation, burning stool) without any urinary signs, the rear-zone finding is Large Intestine, not Bladder or Kidney, regardless of which exact chart you're using.

Zone	Organ
Tip	Heart
Front third (behind tip)	Lung — also the chest/breast region on the front sides (note: mapping is contralateral for breast health — left tongue side = right breast, right side = left breast)
Center	Spleen/Stomach
Sides	Liver/Gallbladder
Rear	Kidney/Bladder/Intestines (Lower Jiao)

Organ-to-tongue correspondences, per Schrier: the Heart opens to the tongue (it's the Heart's sense organ) and governs speech — a stutter or being "tongue-tied" can point to something unsettled with the Heart, framed through the classical image of the Heart as Emperor unable to issue its mandate clearly. The Spleen governs the tongue's muscles and movement (a subtle quiver, distinct from a stronger involuntary "moving tongue," points to Spleen Qi weakness lacking the strength to hold the tongue steady). The Liver governs the tendons and, when disturbed, produces involuntary movement (deviation, stronger shaking) associated with Internal Wind. The Lung nourishes the fluid pathways that keep the tongue's surface properly moist.

The Normal Tongue — "Goldilocks"

Schrier's baseline: pale-red color (not "wow, that's red" — a soft, light pink), fits the mouth neither too big nor too small ("Goldilocks, just right"), not swollen, not thin, no cracks, sits still without flopping or trembling, no ulcers, thin white coating, and slightly moist — like a healthy dog's nose, not bone dry and not dripping. Sublingual veins should not be visibly distended. He's candid that a truly "perfect" normal tongue is rare in practice — "even the Buddha probably has some scalloping, from worrying about all of us."

Tongue Body Color — Building the Pattern Piece by Piece

Schrier's teaching device here is building a pizza: dough, sauce, and cheese give you a base; add pepperoni and you have a pepperoni pizza; add ham and pineapple and you have a Hawaiian pizza. Tongue color diagnosis works the same way — each observation is a topping you add to build the final pattern, not a single fact to memorize in isolation.

Color	Variant	Indication
Pale	Wet + swollen	Yang Deficiency — not enough Yang to mobilize fluids, so they pool and swell the tissue
Pale	Slightly dry, thin	Blood Deficiency (not enough Blood to fill the tongue out) or Yin Deficiency if heat signs accompany it
Pale	Sides only	Liver Blood Deficiency — may show dry eyes, scanty periods, and poor sleep because Blood isn't anchoring the Hun (see box below)
Pale	Center section only	Spleen/Stomach (Earth) Blood Deficiency
Red	With coating	Full-Heat (you're wearing a coat — an extra layer, i.e. an Excess condition)
Red	Without coating	Empty-Heat (you took the coat off — a Deficiency condition)
Red	Tip only	Heart Fire (with coating, a Full condition) or Heart Empty-Heat (peeled, no coating)
Red	Sides w/ coating	Liver Fire, Liver Heat, or Liver-Yang rising (Full condition)
Red	Sides w/o coating	Liver Yin Deficiency with Empty-Heat
Red	Center w/ coating	Full Heat in Spleen/Stomach
Red	Center w/o coating	Spleen/Stomach Yin Deficiency with Empty Heat
Purple	Reddish-purple	Blood stasis arising from Heat
Purple	Bluish-purple	Blood stasis arising from Cold
Purple	Sides	Liver Blood Stasis
Purple	Bluish-purple, sides, in women	Consider Uterus (Liver and Uterus share a close relationship) in addition to Liver
Purple	Center	Spleen/Stomach Blood Stasis
Blue	—	Interior Cold, potentially progressing toward Blood Stasis as the color deepens toward purple

Purple always means Blood Stasis, no exceptions — that's the rule Schrier repeats as a call-and-response drill ("I say purple, you say Blood Stasis"). The shade (reddish vs. bluish) then tells you whether Heat or Cold drove the stasis, and the location tells you which organ.

Balloon-and-string analogy for Blood anchoring the Hun: think of a helium balloon with a weight tied to the string. The weight (Blood) is what keeps the balloon (Hun, the Liver's ethereal soul) from floating away. Too light a weight, and the balloon drifts off — which is why Liver Blood Deficiency so often shows up clinically as poor, unsettled sleep: there isn't enough Blood to anchor the Hun at night.

Tongue Shape

Finding	Pattern
Thin	Blood or Yin Deficiency (pale = Blood def; red without coating = Yin def)
Swollen, pale, wet	Yang Deficiency — not enough Yang activity to transform/transport fluid, so it accumulates as Dampness
Swollen, red	Heat (check for accompanying redness/irritability/thirst to distinguish from a Damp-only swelling)
Tooth-marked / scalloped edges	Spleen Qi Deficiency — the Spleen can't transform/transport, the tongue bloats slightly, and it presses against the teeth. (Not a Liver sign just because it appears on the sides — context and accompanying signs decide.)
Cracks	Dryness — think of soil cracking as it dries; commonly Yin Deficiency, but can also be Dampness or Yuan Qi Deficiency depending on the accompanying picture
Quivering (gentle)	Spleen Qi weakness — the tongue's muscles lack the strength to hold fully steady
Moving (stronger, restless)	Liver Wind — distinct from a quiver in degree, not kind
Deviated (shoots to one side)	Internal Wind (or a structural/physical cause — Schrier notes his own teacher once asked "did you have any accidents?" before concluding Wind, since a prior facial injury can also deviate the tongue)
Stiff / rigid	Internal Wind or Blood Stasis
Flaccid	Deficiency of Body Fluids or Blood Deficiency
Long	Heat
Short	Severe Yang Deficiency (retracted, like curling in from cold) or, less commonly, severe Heat drying and curling the tissue

Coating

Finding	Indication
Thin, white	Normal, or early Exterior condition — check for a Floating pulse before concluding Exterior; if absent, it may simply be a normal coating
Thick, yellow	Pathogen has moved from the Wei (exterior) level toward the Interior; the coating both thickens and yellows as Heat penetrates deeper
Rootless / easily wiped off	Stomach Qi/Yin Deficiency
Peeled, no coating	Yin Deficiency
Sticky	Dampness or Phlegm

Quick Recall (Wen Bing framing): red prickles clustered at the tongue's edges = pathogen still relatively Exterior; redness moving toward the center, mental changes at night, or macular rashes = moving into the Ying (Nutritive) level; frank bleeding signs = the deepest Xue (Blood) level. There's no single hard cutoff line — you layer this against the rest of the four-level picture (is aversion to cold + fever + Floating pulse still present, or gone?).

Sublingual Veins

Finding	Indication
Distended, not dark	Qi Deficiency
Thin	Yin Deficiency
Distended and dark	Blood Stasis in the Upper Burner
Dark and dry	Severe Yin Deficiency with Empty Heat
Dark, swollen, wet	Lung, Spleen, and Kidney Deficiency
Reddish and shiny	Damp-Heat
White and slippery	Cold-Damp

5 • Week 6 — Diagnosis by Observation 望診

Schrier opens this unit with the legend of Biǎn Què, one of the earliest famous physicians in Chinese medical history. As the story goes, Bianque ran a boarding house and, unlike everyone else, took in an elderly sage with no money to pay. Moved by his compassion, the sage gave Bianque a secret formula — herbs mixed with morning dew — that let him "see through walls." Schrier's point: you're not getting X-ray vision from a cereal-box novelty, but you are developing the real version of that power. You've already started doing it with the tongue — seeing a greasy yellow coating and immediately knowing something is off with Spleen/Stomach Damp-Heat. Diagnosis by Observation trains that same seeing-through-the-surface skill on the rest of the body: complexion, eyes, body shape, movement.

Method

Method	Uses
Inspection	Eyes: vitality, color, appearance, secretions (incl. tongue)
Auscultation/Olfaction	Ears (listen) + nose (smell)
Inquiry	Verbal questioning
Palpation	Touch, incl. pulse

Quick Recall: Zang-Xiang = what's inside shows on the outside. The body is a microsystem of microsystems — face, tongue, ear, hand, and foot all independently map the whole body, which is also why distal acupuncture works: needling a point on the hand or foot for a headache is no different in principle than walking across a room to flip a light switch instead of rewiring the ceiling directly above your head.

Shen (Vitality) — Reading the Spark in the Eyes

Sū Wèn 43 states that the Shen is stored in the eyes, and when the Shen becomes restless it withers away and the sparkle fades. Schrier's illustration: even a genuinely unsettling actor like Steve Buscemi, playing an unhinged character, still has a visible twinkle in his eyes on screen — the Shen is still present, just presented oddly. Compare that to the flat, empty stare you see in mugshot photos of figures like Charles Manson or Jeffrey Dahmer, or the "void" eyes horror films use for possessed characters — there the light genuinely isn't there. When Schrier picks produce at the grocery store, he applies the same principle: an orange that looks dull and lifeless doesn't have "Shen," and he'll skip it for a juicier, more vibrant-looking one. The same judgment applies to a patient's overall presence.

Observation	Having Shen	Loss of Shen	False Shen
Complexion	Bright and moist	Dim, dull sheen	Dull, then suddenly lustrous
Eyes	Alert, vivid, twinkle	Dull, blurred vision	Dull, then suddenly bright
Mind/Speech	Clear speech, conscious, agile	Unconscious, delirious	Unclear, then sudden change
Motion/Response	Quick, appropriate	Sluggish and slow	Suddenly responsive
Respiration	Even	Rapid, feeble, or difficult	Feeble, then briefly steady
Appetite	Strong, wants to eat	Anorexic, no desire to eat — may feel like just wanting to lie down and give up	—

False Shen — back to the candle, and the "green flash": Schrier ties False Shen directly back to the Jing-Qi-Shen candle image from Week 1. When the candle's wax (Jing) is nearly gone and the flame (Qi) is guttering low, there's sometimes one sudden, brighter flare right before it goes out completely — like the "green flash" some sailors describe seeing right at the moment the sun disappears below the horizon. He recounts a family story: a dying relative was waiting for one more sibling to arrive before she would let go. When that sibling finally walked in, she suddenly became fully alert, spoke clearly (in this case, to angrily call them out for being late), and passed away shortly after. The lesson holds whether the last moment is angry or tender: False Shen is the fire flaring one last time before going out, not a genuine turnaround.

Deranged Shen Patterns

Term	Presentation
Fán zào	Irritability with hot/feverish sensation; may drive the patient to prefer nudity or jump into water
Zàng zào	Visceral agitation — fluctuating rapidly between happy and very sad, crying spells, frequent yawning, disorientation
Bǎi hé bìng (Lily disease)	Chronic depression, apathy, aversion to speaking/mumbling to self, insomnia, anorexia, sleepwalking
Diān (Yin madness)	Inward-turning: chronic depression, aversion to speaking or mumbling to self, inappropriate laughing/crying
Kuáng (Yang madness)	Outward-turning: raving with obscenities, restlessness, singing loudly in high places, wandering naked, aggression

Schrier connects some of these presentations to lifestyle causes seen in populations with heavy processed/fast-food diets and alcohol use: greasy, fried food generates Phlegm that can cloud the sensory orifices, while alcohol generates internal Heat that rises and can aggravate agitation — not exclusive to any one population, but a pattern he's observed clinically.

Su Wen 32 — Face Heat Map

Schrier's holiday-season mnemonic: "why does Rudolph have a red nose? Why does Santa have a red nose?" — too many cookies (Spleen-associated Heat, since the nose maps to Spleen), and maybe something a bit stronger than milk. The joke sticks because it's the same logic you'll use clinically: a red nose points you toward Spleen Heat, and you ask about diet (sweet, greasy, or spicy intake) accordingly.

Region (patient's own left/right)	Organ
L cheek (patient's)	Liver
Forehead	Heart
Nose	Spleen
R cheek (patient's)	Lung
Chin	Kidney

The mirror trap: Schrier flags this as a near-guaranteed quiz and final-exam item precisely because it's so easy to get backwards. When you draw the face map facing yourself (as if looking in a mirror or at a diagram), Liver appears on your left as you look at the page — but that's the *patient's* left cheek, not yours. Schrier admits he's marked his own quizzes wrong on this because his brain defaults to the diagram orientation. In clinical heat disease, the Liver-associated cheek reddens first for Liver Heat; the forehead reddens first for Heart Heat.

Quick Recall: different classical sources map the face differently (Ling Shu 49 places the bridge of the nose as Liver rather than Spleen) — treat these as different systems, not contradictions, the same way different pulse or tongue-map traditions coexist.

Five Colors of Face

Color	Indication
Red	Heat (excess=whole face; malar only=Yin-def heat)
Pale/White	Cold, blood/yang deficiency (puffy=yang qi def; withered=blood def)
Yellow	Deficiency, damp (bright=yang jaundice/damp-heat; smoky=yin jaundice)
Blue-Green	Cold, pain, stagnation, convulsion
Dark Grey/Black	Kidney deficiency, blood stasis

Quick Recall: lustrous complexion = good prognosis; dark/haggard = poor prognosis. As with pulse and tongue, take facial color readings in natural light, ideally before the patient has rushed in flustered — a patient sprinting to make their appointment will show a temporarily flushed face that reflects exertion, not an underlying Heat pattern. Let them settle first.

Body Shape & Movement

Finding	Pattern
Overweight + depression	Qi deficiency + excess phlegm-damp
Thin, dry skin	Blood insufficiency
Great weight loss, long illness	Exhaustion of essential Qi
Overweight overall	Yang deficiency with phlegm
Thin overall	Yin deficiency
Active movement	Yang syndrome
Passive manner	Yin syndrome
Violent limb movement	Wind disease (tetanus, infantile convulsion)
Muscle weakness/atrophy	Wei syndrome
Pain+swelling+restricted joints	Bi syndrome (painful obstruction)

Five Wheels of the Eye

Wheel	Structure	Organ
Blood	Inner/outer canthus	Heart
Qi	Sclera	Lung
Wind	Iris	Liver
Water	Pupil	Kidney
Flesh	Eyelids (upper=Spleen, lower=Stomach)	Spleen/Stomach

6 · Week 7 — Diagnosis by Inquiry 問診

Schrier opens by naming a historical precedent for tailoring the question order: Zhāng Zhòngjǐng always started with Cold and Fever — essentially asking "is this an Exterior or an Interior pattern?" before anything else. Schrier's own approach is looser: follow the patient's chief complaint first, then let that complaint's logic dictate which of the 16 Questions actually matter. A straightforward musculoskeletal injury doesn't need an exhaustive dietary interrogation the way a case of stomach cramps and acid reflux obviously does — though he still holds that diet is worth asking about in nearly every case, "because what are you feeding your muscles? What are you feeding your bones?"

The 16 Questions — Asked Loosely, Not Rigidly

Topic	Detail
10 vs 16 Questions	Maciocia's 10: chills/fever, sweating, head/body, 2 excretions, diet, chest/abdomen, hearing, thirst/drink, previous illness, cause. Schrier's 16 adds Pain, Emotions, Sexual Symptoms, Energy Level.
16 in order	1 Pain · 2 Food/Taste · 3 Stools/Urine · 4 Thirst/Drink · 5 Energy · 6 Head/Face/Body · 7 Chest/Abdomen · 8 Limbs · 9 Sleep · 10 Sweating · 11 Ears/Eyes · 12 Cold/Heat/Fever · 13 Emotional · 14 Sexual · 15 Women's · 16 Children's
Golden Rule	Every question should confirm or exclude a pattern — never recite the 16 as a rote script; always ask why. Ask about family/ social life and sleep without being invasive — you're gathering the emotional and lifestyle context (work hours, caregiving burden, commute) that often explains why a pattern developed in the first place.

Schrier illustrates why lifestyle questions matter with a deliberately exaggerated commuter example: someone caring for an ill parent who commutes hours each way, works all day, comes home to caregiving duties, and finally gets to bed near midnight only to wake at 3am to do it again — that schedule alone explains a Qi and Blood deficiency picture without needing any other cause. He also encourages asking what's going well in the patient's life, not only what's wrong, since practitioners can otherwise slide into only ever hearing the negative.

Q1 — Pain

Topic	Detail
3 mechanisms (Neijing)	Circulation impeded (Su Wen 39) · Tension (Su Wen 39/62, Ling Shu 10) · Lack of nourishment
Full vs Empty pain	Full: aggravated by food/BM/vomiting/pressure, relieved by rest, sharp, worse w/ movement. Empty: alleviated by food/BM/ rest/pressure, dull, better w/ movement.
Pain character	Distending/moving=Qi stagnation · boring/stabbing fixed=Blood stasis · cramping+cold=Cold · burning+fullness=Heat · intense+fullness=food retention

Wandering Bi — the hot dog cart analogy: Schrier's image for Wind-type Bi Syndrome: paying a food-truck vendor on a windy day and your money blows out of your hand — it drifts one way, you chase it, it flips another way. That's Wind: pain that isn't anchored anywhere, moving joint to joint. Compare that to Painful Bi (severe, fixed pain from Cold), Fixed Bi (soreness/heaviness from Damp), Heat Bi (sudden swelling and redness), and Bone Bi (chronic, deep, a progression of the other four).

Bi Syndrome	Pathogen	Character
Wandering	Wind	Moves joint to joint, nothing fixed
Painful	Cold	Severe, fixed pain
Fixed	Damp	Soreness and heaviness
Heat	Heat	Sudden onset, swelling, redness
Bone	Chronic, deep	Progression of the other four; joint deformity

Q2–4 — Food, Taste, Stools, Urine, Thirst

Schrier widens "appetite" beyond food itself: the Stomach's job is reception — receiving food and drink, but by extension, receiving information. A student who's lost interest in the material, or who can't tolerate a particular environment, is showing the same "poor reception" logic in a different domain. Ravenous appetite doesn't automatically mean Stomach Fire, either — he flags parasites (tapeworms) as a real differential to rule out before settling on a Heat diagnosis, and reminds students to check for the fuller Stomach-Heat picture (burning sensation, acid regurgitation, red tongue redder in the center, possible central cracking) before committing.

Topic	Mapping
Appetite	Lack=SP-Qi def · excessive=ST-Heat (rule out parasites first) · fullness=Dampness/food retention · distention=Qi stagnation
Taste	Bitter=Liver/Heart Heat · sweet=SP def or Damp-Heat · sour=food retention or LR invading ST · salty=KD-Yin def · pungent=LU-Heat
Stools	Better after BM=Full · worse after BM=Empty · undigested food=SP-Qi def · black/dark=Blood stasis · alternating constip/diarrhea=stagnant LR-Qi invading SP
Urine color	Pale=Cold · dark=Heat · turbid/cloudy=Dampness
Urine pain timing	Before=Qi stagnation Lower Burner · during=Damp-Heat Bladder · after=Qi deficiency
Thirst	Cold drinks desired=Heat · warm drinks desired=Cold · no thirst=Cold/no Heat · thirst but no desire to drink=Damp-Heat or Yin def

Q5, Q9, Q10 — Energy, Sleep, Sweating

"I'm so tired" is, in Schrier's words, not enough information — "tell me more" is doing most of the diagnostic work here. He contrasts two tiredness pictures that patients might otherwise describe the same way: chronic tiredness with a desire to lie down, poor appetite, and loose stools points toward Spleen (or Spleen-Yang) Deficiency, while chronic tiredness with a weak voice and a tendency to catch colds easily points toward Lung Qi Deficiency instead — because Lung and Spleen are the two organs that generate postnatal Qi (Lung from air, Spleen from food/drink), and the accompanying signs tell you which one is actually short.

Topic	Mapping
Tiredness	AM worse=SP/ST-Yang def · weak voice+catches colds=LU-Qi def · feeling cold=KD-Yang def · heaviness=Dampness · muzy+dizzy=Phlegm · distention+irritable+Wiry=LR-Qi stagnation
Sleep physiology	Depends on Blood/Yin of Heart & Liver; Spleen also influences. Shen and Hun need anchoring by HT/LR Blood.
Insomnia types	Can't fall/stay asleep=Blood def not anchoring Shen/Hun (Empty). Restless+heat+dreaming=pathogens agitating Shen/Hun (Full). Wake many times=Yin def w/ Empty Heat. Wake early=GB deficiency.
Sweat location	Head only=ST Heat/Damp-Heat · whole body=LU-Qi def · palms/soles/chest=Yin def (5-palm heat) · day=Yang def · night=Yin def

Not tested: Categories of Dreams / Neijing dream chapters (Su Wen 17, 80; Ling Shu 43) are explicitly marked not tested, even though Schrier notes dream content can theoretically point toward specific organs.

Q6 — Head, Face, Body

Schrier repeats the mint-tea caveat from Week 1 here in the specific context of headaches: mint tea disperses Liver-Qi stagnation and helps an Excess-type headache, but the same cooling, dispersing action will make a Deficiency-type headache worse. You have to know which kind you're looking at before recommending it.

Headache Location	Channel	Cause
Nape	Tai Yang	Ext. Wind-Cold or Kidney def
Forehead	Yang Ming	Stomach-Heat or Blood def
Temples/sides	Shao Yang	Ext. Wind or Liver/GB Fire rising
Vertex	Jue Yin	Liver-Blood deficiency

Headache Timing/ Quality	Indication
Daytime	Qi/Yang deficiency
Evening	Blood/Yin deficiency
Night	Blood stasis
Sudden onset, severe giddiness, everything sways	Internal Wind
Throbbing, heaviness, muzzy-headed	Liver Yang rising
Slight dizziness, worse with fatigue	Deficiency (vs. sudden/severe onset = Excess)
Numbness: bilateral vs unilateral	Bilateral=Blood deficiency; Unilateral (esp. first 3 fingers)=Internal Wind + Phlegm, possible impending Wind-stroke

Face Pain and Nosebleeds — Reading the Same Region Through Different Lenses

Schrier uses facial pain to show how the same location can point to different organs depending on the accompanying picture. Forehead pain with red cheeks, thirst, and a bitter taste could be Liver Fire — but the forehead also corresponds to Yang Ming/Stomach, so the same location could instead mean excess Fire invading the Stomach (digestive signs) or the Heart (mental/emotional signs). The differentiator is always the accompanying symptom set, not the location alone.

Bleeding Mechanism	Accompanying Signs	Pattern
Excess pushing blood out	Feeling of heat, thirst, inflamed gums	Fire/Heat (Excess)
Deficiency, not holding	Tiredness, poor appetite, loose stools	Spleen Qi not holding Blood (Deficiency)
Deficiency Heat	Dry mouth, desire to drink in small sips (not gulping)	Yin Deficiency with Empty Heat

Quick Recall: bleeding has three root mechanisms — too much Heat pushing Blood out, too much tension/pressure forcing it out, or a Deficiency that fails to hold it in. Ask which one before treating the bleeding itself.

7 · Week 8 — Diagnosis by Palpation 切診 (non-pulse)

Schrier frames palpation as giving you access to information the other three pillars can't: the state of the channels themselves, trigger points, adhesions, and structural findings you simply cannot see or ask about. His teaching method for this unit is heavily hands-on — students spend much of the class actually palpating each other's forearms, lower legs, and abdomens rather than only reviewing slides.

Channel Palpation — the Beaver Dam and the Gummy Bear

Schrier compares an obstructed channel to a beaver dam: Qi backs up behind the blockage, and when you release it (needling the point), the pent-up flow can come through forcefully enough that a patient feels a jolt somewhere distant — the classic experience of needling the arm and feeling it in the opposite-side toe, following the paired six-conformation relationships. For nodules found along a channel, his practical texture test is gummy bears: is it soft and squishy (fresher, more moist) or hard and stale, like a gummy bear left out of the package on the counter too long? That texture distinction maps onto Phlegm (softer) versus Blood Stasis or Cold (harder, more congealed).

Technique	Method	Finding → Meaning
Channel tracing	Palpate forearm (wrist→elbow) and lower leg (ankle→knee) three times, pressing progressively deeper each pass — light, medium, deep (Heaven, Earth, Humanity)	Structural changes, muscle tension, nodules, lumpiness along the pathway
Pressing (Ashi)	Thumb/fingertip pressure along the channel and at points	Tenderness/pain = Excess. Feels good under pressure = still some stagnation, but with an underlying Deficiency component — "the more it feels good, the more something's stuck that wants to move." Slippery or hard = Phlegm.
Feeling	Temperature, moisture, skin texture	Rough (like a cat's tongue or lizard skin), sticky/gummy, or the presence of rashes along a channel's course all localize to that channel's associated organ or region

Quick Recall: palpate moving upward along the direction of the Five Transport (Wu Shu) points — from fingers/toes toward elbows/knees — not downward; this is a diagnostic convention distinct from Qigong energy work, which can move either direction.

Skin Temperature — Why the Back of the Hand Goes First

Schrier's practical tip: check temperature with the back of your hand before the palm, because the back of the hand runs cooler and gives you a more accurate baseline reading rather than one skewed by your own palm's warmth. He also cautions that clothing and room temperature can distort the reading — a patient wearing a heavy sweatshirt on one side of the body, or sitting in a cold exam room, can present a temperature difference that has nothing to do with their underlying pattern. Always account for the room and the layers before concluding anything.

Presentation	Pattern
Cold	Cold pattern
Hot on 1st touch, cools w/ sustained pressure	Exterior Wind-Heat, still only at the Wei (exterior) level
Hot over the blood vessel on medium pressure, not heavy	Interior Heat in the Middle Burner/Heart
Hot on heavy pressure, nearly reaching the bone	Empty-Heat from Yin Deficiency — the extreme form of this is "steaming bone disorder," where heat feels like it's radiating from the bone itself
Hot dorsal (back of hand)	Full Heat (Ling Shu 74)
Hot palm	Empty Heat (Ling Shu 74)

Hands and Feet

Finding	Pattern
Cold hands and feet, warm torso	Liver Qi Stagnation — the Liver isn't ensuring smooth Qi flow to the extremities, so warmth stays trapped centrally
Cold hands	Lung and/or Heart Yang Deficiency
Cold feet	Kidney (Yang) Deficiency
Cold feet, in women specifically	Liver Blood Deficiency
Cold fingers and toes	Liver Qi Stagnation

Xu Li (Apical Beat) — The Pool Ladder Test

The apical beat is the pulse of the heart/left ventricle, felt at the fifth intercostal space. Schrier's analogy for the region just below the xiphoid process (reflecting the Upper Burner and Zong Qi): a set of pool ladder steps. If you press your thumb straight through a worn stair tread with no resistance, that structure has failed — too soft, too giving, which maps to Deficiency. A healthy structure has just enough give without being rigid — too hard and unyielding maps to Full/stagnation.

Presentation	Pattern
Clear/slow	Normal
Feeble	Zong Qi deficiency
Strong/hard	Excess Lung/Heart
Large but empty	Heart-Qi deficiency
Stop-start	Severe shock or alcoholism; poor prognosis if discrepant from the radial pulse
Rapid	Shock, fright, or sudden anger

Abdominal & Point Palpation

Schrier traces the abdominal map to the same cosmological Five-Phase layout used for the face and pulse, but notes it wasn't fully developed until Japanese practitioners — particularly the blind acupuncturist Waichi Sugiyama — extended the foundational work found in the Nan Jing and Shang Han Lun. Sugiyama is also credited (per the story Schrier tells, acknowledging it's more legend than verified fact) with inventing the guide tube after a fall down a ravine left a pine needle wrapped in a leaf pressed against his skin — giving him the idea for a way to insert needles with far less pain than his own famously rough technique. Practical courtesy point: never palpate straight down a patient's abdomen from the pubic bone without warning — ask them to place their own hand at the top of the pubic bone (CV3) as a landmark, and work from CV17 downward to the umbilicus, and from CV3 upward to the umbilicus, rather than palpating across that boundary directly.

Topic	Detail
Nan Jing 5-Phase map	Above navel (Ren 12–15)=Heart/Fire · at navel=Spleen/Earth · below navel (Ren 7–Pubis)=Kidney/Water · R side under ribs=Lung/Metal · L side under ribs=Liver/Wood
Normal abdomen	Solid-not-hard, resilient-not-tight, elastic-not-soft (the "pool ladder" test). Hard+pain worse w/ pressure=Full. Too-soft+ache relieved=Empty. Fixed masses=Blood stasis.
Palpating lumps (channel, breast-adjacent, abdominal)	Soft=Phlegm · Hard=Blood Stasis or Cold congealing · Distinct edges=Phlegm · Indistinct edges=Toxic Heat · Mobile=Phlegm-Damp · Immobile=Blood Stasis or Toxic Heat
Front-Mu vs Back-Shu	Front-Mu (alarm points) = where Qi of the organs collects/accumulates, more associated with treating Excess. Back-Shu = stronger Qi reserve, often better for Deficiency, though also used to clear Excess.
Front-Mu numbering mnemonic	Higher rib-cage points correspond to upper-body organs and lower-numbered points near the sternum/chest; a quick way to sanity-check a Back-Shu point's location is that lower numbers (BL 13) sit higher on the back (Lung) and higher numbers (BL 28) sit lower (Bladder) — upper, middle, and lower Burner order top to bottom.

Aggressive Energy (Worsley system): a non-classical addition Schrier includes because some students will encounter it in Five Element/Worsley-influenced clinics. AE is described as toxic Qi circulating through the Ke (control) cycle; under the fingertip it reportedly feels like the buzzing vibration of an electric razor at specific Back-Shu points (BL 13, 14, 15, 18, 20, 23). Diagnosis is confirmed only through the treatment itself — needling the point plus a nearby "test needle" and watching for erythema (redness) that lingers around the main needle longer than around the test needle. This is not on the national boards; it's included as clinical color for those who go on to study Five Element/Worsley-style acupuncture.

Palpating the Chest

Finding	Pattern
Tender at CV17 with light palpation	Heart Blood Stasis (or, depending on location, Lung Excess)
Tenderness relieved by light palpation	Deficiency
Tenderness relieved by light palpation but elicited by deeper palpation	Mixed Deficiency and Excess
Tenderness increases with deeper palpation	Excess

Palpating the Three Jiao (Burners)

The goal isn't necessarily to find "normal" (evenly warm) — Schrier's clinical preference is actually for a uniform temperature across all three Burners, even if that uniform temperature is cold or hot throughout, over a lopsided picture (e.g., a very cold Lower Burner beneath a hot Middle and Upper Burner). A uniform imbalance is, paradoxically, more internally consistent and often more straightforward to treat than a split thermostat within the same body.

Burner	Location	Contains
Upper	Above the diaphragm	Heart, Pericardium, Lungs
Middle	Between diaphragm and navel	Stomach, Spleen, Liver, Gallbladder
Lower	Below the navel	Small Intestine, Large Intestine, Bladder, Kidney

8 · Week 9 — Diagnosis by Hearing & Smelling 聞診

Wén Zhén is the second of the Four Diagnostic Methods (望聞問切) and has two distinct components sharing one character (聞 — "to perceive with the ear/nose"). Schrier is explicit about what you're actually doing with the smelling component in practice: you are not smelling a patient's urine, stool, sputum, or vaginal discharge. You ask them what these things smell like, since they have direct access to that information and you generally don't (sweat and breath being the practical exceptions you might notice directly in the room).

Component	Covers
Hearing	Voice, breathing, cough, vomiting, hiccup, borborygmi, sighing, belching — any sound emitted by the patient
Smelling	Body odor, and odor of bodily secretions (breath, sweat, sputum, urine/stools, vaginal discharge, intestinal gas)

Voice & Zang-Fu Relationship

Schrier separates the voice's production from its volume: the Lung is the gateway of the voice, moving Qi to create sound in the first place, while the Kidney is the root of the voice, controlling volume because it's the root of Yuan Qi and governs the Qi's reception once the Lung sends it down. He offers a "Jimmy" story to illustrate Wood/shouting-style assertiveness: a substitute teacher, trying to get his attention, kept calling him by the wrong name ("Jimmy") with increasing frustration, eventually smacking the desk to demand a response — assertive, forceful, attention-demanding, without necessarily being angry. That forcefulness (not necessarily volume alone) is the signature of the Wood/Shouting sound.

Organ	Role	Governs
Lung	Gateway of the voice; movement of Qi helps create sounds	Production of sound
Kidney	Root of the voice; root of Yuan Qi	Volume of the voice
Spleen/Stomach	Acquired root of Qi	Strength of the voice

Voice, Breathing & Reflexive Sound Patterns

Schrier tells a story about his cat, nicknamed "Darth Vader" for her labored, raspy breathing, caused (it turned out, after years of tests) by a large nasal polyp; once it was surgically removed, the sound disappeared entirely. The point for clinical practice: an abnormal respiratory sound is real diagnostic data even when its cause isn't obvious right away, and structural causes should stay on the differential alongside pattern-based ones.

Presentation	Pattern
Loud voice	Full or Heat pattern — think of how your own voice sounds when you're truly angry and trying to make someone stop: forceful, loud, hot
Weak voice	Empty or Cold pattern
Reluctance to talk	Cold pattern or Lung-Qi deficiency — Cold slows everything down, including the will to speak
Excessive talking	Heat — Schrier's example: children as "little hot buckets," constantly narrating everything ("did you know I can stand on my toes?"); adults who talk excessively often report feeling warm or overheated while doing it
Sudden loss of voice	Invasion of Wind-Heat
Gradual loss of voice	Lung-Qi or Lung-Yin deficiency
Coarse, loud breathing	Full pattern
Weak, thin breathing	Empty pattern
Loud, explosive cough	Full pattern
Weak cough	Empty pattern
Borborygmi (stomach gurgling)	Loud=Full; low/weak=Empty (Spleen/Kidney Yang deficiency)

Five Element Sounds — With Cultural Illustrations

Schrier uses broad, explicitly non-judgmental cultural comparisons to make these sounds memorable: he describes German speech patterns as leaning Wood/assertive (direct, forceful, even in affectionate expressions), Welsh speech as carrying a Fire-like rising-and-falling melodic quality, and Japanese cultural precision (tea ceremony, sword forms, bento box arrangement) as exemplifying Metal's clean, exact, single-motion perfection. These are teaching illustrations of the sound-quality associations, not clinical claims about any individual patient's constitution.

Sound	Element	Quality
Shouting	Wood	Forceful, attention-demanding, assertive (not necessarily angry)
Laughing	Fire	Outward, bursting expression — including the "pre-laugh" giggle a child gives before they even finish telling a joke
Singing	Earth	Soothing, nurturing, rises and falls gently — the voice you use calming a child or an anxious animal
Weeping	Metal	Consumes/dissolves Qi; a voice on the verge of tears, choking back words
Groaning	Water	Flat, stretched, little modulation — like a VHS tape played back slowed down

Diagnosis by Smelling

Odor	Description / Indication
Rancid/Sour	Fat no longer fresh; also smell of newly-mown grass; prickling effect on the nose
Scorched	Burnt toast, clothes from the dryer, a scorched shirt while ironing
Fragrant	Related to pleasant smells like flowers, but not as pleasing — cloying, sickly sweet, tends to linger
Rotten	Rotting meat; fills the nose with tiny prickles
Putrid	Similar to rotting meat but sharper/more aggressive; smell of stagnant water

Schrier's distinction for a true Cold-Damp "fishy" odor: not the aggressive smell of a Chinatown fish market, but the very light, mild, faintly salty smell of a freshly caught fish held up and sniffed directly — subtle, not pungent. Strong, intense odor generally points toward Heat/Excess; faint or absent odor toward Cold/Deficiency. He repeats the gravy-thickening-on-the-stove image from earlier weeks for the consistency of vaginal discharge/leucorrhea: watery (thin, like gravy mix fresh in cold water) points to Cold/Deficiency, while thick and sticky (like gravy reduced too long on the heat) points to Heat/Damp.

Secretion	Finding	Pattern
Breath	Strong, foul	Stomach Heat or retention of food
Sweat	Putrid-smelling	Damp-Heat
Sputum	Strong-smelling, rotten	Phlegm-Heat or Toxic Heat
Sputum	Fishy-smelling	Lung-Heat
Urine	Heavy smell of urea, scanty yellow	Heart fire pouring into Bladder
Urine	Clear, frequent, odorless	Cold
Vaginal discharge	Strong-smelling, leathery, thick/sticky	Damp-Heat
Vaginal discharge	Fishy smell, thin/watery	Damp-Cold

Flag: the Scorched odor slide uses 笑 (xiào) where the classical character is 焦 (jiāo) — confirm with Prof. Schrier before the final. The Five Named Odors are not explicitly paired to Zang-Fu organs in the source slides.